

Trial		Characteristics		Patient Selection			Comparator			Primary Outcomes				Safety Outcomes				Treatment Effects				Setting	
Trial	Year	N	Time Window	Agent	Occlusion	EVT	Control	Antiplatelet*	Ctrl EVT	Imaging	Outcome	Treatm ent	Control	P-value	sICH Tx	sICH Ctrl	Mort Tx	Mort Ctrl	Effect Size	95% CI	ARR		Region
OPTION	2026	570	4.5-24h	TNK 0.25 mg/kg	Non-LVO (~74% MeVO/distal)	No planned EVT	Std care (antiplatelet + guidelines) Std treatment (antiplatelet + support)	Ctrl: from randomization; TNK timing not explicit Ctrl: from randomization; tPA arm: after 24h	Rescue 6/282 (2.1%) TNK, 1/284 (0.4%) ctrl	CTP mismatch	mRS 0-1	43.6%	34.2%	0.020	2.8%	0.0%	5.0%	3.2%	aRR 1.28	1.04-1.57	9.4%		China
HOPE	2025	372	4.5-24h	tPA 0.9 mg/kg	~63% LVO	No planned EVT			Rescue 7/186 (3.8%)		mRS 0-1	40.3%	26.3%	0.004	3.8%	0.5%	10.8%	10.8%	aRR 1.52	1.14-2.02	14.0%		China
EXPECTS	2025	234	4.5-24h	tPA 0.9 mg/kg	Posterior	No planned EVT	Std care + antiplatelet	43.5% mono, 56.5% dual	Rescue 7/117 (6.0%)	hypodensity	mRS 0-2†	89.6%	72.6%	0.010	1.7%	0.9%	5.2%	8.5%	aRR 1.16	1.03-1.30	17.0%		China
CHABLIS-T II	2025	224	4.5-24h	TNK 0.25 mg/kg		54.9% EVT	Best medical tx (IV tPA OK)	NR	23.1% tPA + 56.6% EVT	Perfusion CT	Reperfusion w/o sICH	33.3%	10.8%	0.001	5.4%	4.4%	10.8%	10.6%	aRR 3.0	1.6-5.7	22.5%		China
TRACE-III	2024	516	4.5-24h	TNK 0.25 mg/kg	100% LVO	No planned EVT	Std medical tx (antiplatelet, inv. disc.)	At inv. discretion	Rescue 5/252 (2.0%)	CTP or MRI perfusion	mRS 0-1	33.0%	24.2%	0.030	3.0%	0.8%	13.3%	13.1%	Rel. rate 1.37	1.04-1.81	8.8%		China
TIMELESS	2024	458	4.5-24h	TNK 0.25 mg/kg	100% LVO	77.3% EVT	Placebo + std medical tx	NR	178/230 (77.4%) EVT	CTP or MRI perfusion	Ordinal mRS	3	3	0.450	3.2%	2.3%	19.7%	18.2%	Adj. common OR 1.13	0.82-1.57	—		US/Canada
TWIST	2023	578	Wake-up	TNK 0.25 mg/kg	Unselected (LVO 30.0/36.7%)	6% tx; 14% ctrl EVT not part of protocol	No thrombolysis (open-label)	NR	42/290 (14%) EVT	Non-contrast CT	Ordinal mRS	2	2	0.270	2.0%	1.0%	10.0%	8.0%	Adj. OR 1.18	0.88-1.58	—		Europe/NZ
EXTEND	2019	225	4.5-9h	tPA 0.9 mg/kg	70.7% LVO		Placebo + std care	NR	Not specified	Perfusion imaging	mRS 0-1	35.4%	29.5%	0.040	6.2%	0.9%	11.5%	8.9%	aRR 1.44	1.01-2.06	5.9%		Aus/NZ/Fin/TW
ECASS4-EXTEND	2019	119	4.5-9h	tPA 0.9 mg/kg	Not specified	Not reported	Placebo + std care	NR	Not specified	MRI mismatch	Ordinal mRS shift	NR	NR	0.580	1.6%	0.0%	11.5%	6.8%	OR 1.20	0.63-2.27	—		Europe
WAKE-UP	2018	503	Unknown	tPA 0.9 mg/kg	34% intracranial occlusion	Planned EVT excluded	Placebo + std care	NR	Planned EVT excluded	DWI-FLAIR mismatch	mRS 0-1	53.3%	41.8%	0.020	2.0%	0.4%	4.1%	1.2%	aOR 1.61	1.09-2.36	11.5%		Europe
EPITHET	2008	101	3-6h	tPA 0.9 mg/kg	54% arterial obstruction	Pre-EVT era	Placebo + std care	NR	Not specified		Infarct growth	—	—	0.239	7.7%	0.0%	25.5%	14.3%	Ratio 0.69	0.38-1.28	—		Aus/NZ/Bel/UK

Source-qualified teaching update | 30 September 2026

Trial populations must remain separate: WAKE-UP studied MRI DWI-FLAIR mismatch of unknown onset and excluded planned EVT; EPITHET did not require mismatch in every participant; EXTEND allowed CT or MRI perfusion and wake-up midpoint selection. CHABLIS-TII included large/medium occlusions and used a 24-48-hour reperfusion-without-sICH endpoint, unlike 90-day functional outcomes. HOPE used core <=70 mL, ratio >=1.2 and mismatch >=10 mL. Unqualified cross-trial NNT/NNH and population-to-benefit shortcuts are removed; nonsignificant safety differences do not establish harm.

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